coursera

Anxiety and Related Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Components of Anxiety
- ✓ Pause and Reflect: An Angry Dog
  Practice Assignment • Grade: 100%
- ✓ Diverse Manifestations of Anxiety Disorders
  Reading • 3 min
- ✓ Manifestations of Intense Anxiety
  Video • 2 min
- ✓ Check Your Understanding: Components of Anxiety
  Practice Assignment • Grade: 100%

Autonomic Nervous System
- ✓ Components of the Human Nervous System
  Video • 3 min
- ✓ Organs and Internal Systems Involved in Anxiety
  Video • 2 min
- ✓ Going Up the Sky Tower
  Video • 2 min
- ✓ The Components of the Autonomic Nervous System
  Reading • 1 min
- ✓ Check Your Understanding: Autonomic Nervous System
  Practice Assignment • Grade: 100%
- ✓ Activity: Components of Anxiety
  Practice Assignment • Grade: 100%

Key Takeaways
- ✓ Key Takeaways: Components of Anxiety
  Reading • 10 min

Module 4 Quiz
- ✓ Module Quiz: Components of Anxiety
  Graded Assignment • Grade: 90%

Module 5
Disorders that Involve Anxiety Part 1

Generalized Anxiety Disorders
- ○ **Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Generalized Anxiety Disorder**
  Reading • 10 min
- ○ Check Your Understanding: Generalized Anxiety Disorder
  Practice Assignment • 5 min

Panic Disorder
- ○ Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Panic Disorder
  Reading • 10 min
- ○ Check Your Understanding: Panic Disorder
  Practice Assignment • 5 min

Specific Phobias
- ○ Diagnostic Criteria and Prevalence for Specific Phobias
  Reading • 15 min
- ○ Contributing Factors for Specific Phobias
  Reading • 10 min
- ○ Classical Conditioning of Specific Phobias
  Video • 2 min
- ○ Operant Conditioning and Negative Reinforcement of Specific Phobias
  Video • 3 min
- ○ Cultural Issues for Specific Phobias
  Reading • 2 min
- ○ Check Your Understanding: Specific Phobias
  Practice Assignment • 6 min

Key Takeaways
- ○ Key Takeaways: Disorders that Involve Anxiety Part 1
  Reading • 10 min

Module 5 Quiz
- ○ Module Quiz: Disorders that Involve Anxiety Part 1
  Graded Assignment • 10 min

Module 6
Disorders that Involve Anxiety Part 2

Module 7
Conclusion

Module 8
Cumulative Quiz

Module 9
Course Resources

Module 10
PsycLearn Essentials APA Student Resources

Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Generalized Anxiety Disorder

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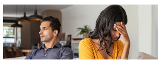


Diagnostic Criteria

A person with **generalized anxiety disorder (GAD)** experiences general apprehensiveness (also called free-floating anxiety) or excessive worry that negative events will occur in various areas of life such as work, school, health, family and relationships (APA, 2022; Crocq, 2017; WHO, 2019). These symptoms need to occur more days than not and have been present for at least three to four months, though the DSM indicates the symptoms need to be present for at least six months. The psychological symptoms (such as difficulty concentrating) are accompanied by emotional symptoms (such as irritability), physiological symptoms (such as overactivity of the sympathetic nervous system resulting in heart palpitations, sweating, dry mouth, gastrointestinal symptoms), and behavioral symptoms (such as muscle tension, restlessness, trouble sleeping; APA, 2022; WHO, 2019). For example, a person with GAD may feel constantly anxious and worried, and may not be able to remember a time when they felt free from anxiety. This anxiety is generalized; this person would not be worried about a single upcoming event or threat, but instead would feel anxious about whatever they are currently focused on.

As you have read, some anxiety is typical, and can even be helpful. However, for people with GAD, their worries are very often out of line with reality: one study showed that 94% of the worries that people with GAD had did not come true (LaFreniere & Newman, 2020). This sort of uncontrollable worrying about events that most likely will not happen is typical of people experiencing GAD.

A complete picture of GAD includes its high likelihood of diagnosis along with other psychological disorders (its comorbidity) that occur at the same time or at other points in the lifespan (Noyes, 2001). The most common comorbid diagnoses are other anxiety disorders, especially panic disorder, and major depressive disorder. Unfortunately, because these diagnoses can reinforce each other, they often make treatments less effective and complicate recovery. For example, in a person with GAD and major depressive disorder, feelings of depression can lead to outcomes that result in anxiety, and suffering from frequent and continual anxiety can be depressing.

Prevalence

The global lifetime prevalence (the proportion of a population that will have a disorder at some point in their life) of GAD appears to be 4%-5% (Kessler et al., 2012; Zuberi, 2021), though in some countries it has been found to be as low as 1.5% (Chang et al., 2019; Mohammadi et al., 2020). GAD is twice as common in women as in men (Crocq, 2017). Prevalence is relatively rare among adolescents and young adults but increases with age and is highest for those over 40 years.

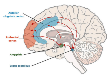
Contributing Factors

Biological Factors in Developing GAD

Much research has examined biological differences between people who have GAD and those who do not. However, findings in this area are often inconsistent (Maron & Nutt, 2017). A few studies have found that GAD is associated with a larger volume of the amygdala—an area of the brain associated with the generation and experience of fear, along with differences in the locus coeruleus, an area that can intensify fear effects (Etkin et al., 2009; Makovac et al., 2016). Other studies have used functional neuroimaging techniques like fMRI to show that patients with GAD display greater neural activity in areas such as the anterior cingulate cortex (ACC) and the prefrontal cortex (PFC), an area involved in the regulation of emotion; along with nearly a dozen others (Blair et al., 2008; Lai, 2019; Paulesu et al., 2010).

Recently, psychologists have added a second approach to understanding how the brain processes anxiety. Instead of focusing on single brain areas, this research focuses on brain circuits, which are networks of interconnected areas involved in perceiving, processing, and responding to anxiety-relevant information (see Duval et al., 2015). Collectively, the areas described above are referred to as the **fear circuit** because they are devoted to processing fear- and anxiety-relevant stimuli. Because of the important roles that the neurotransmitters noradrenaline (Charney et al., 1987) and serotonin (Maron & Shlik, 2006) play in the fear circuit, many psychopharmacological treatments for anxiety in general and GAD in particular attempt to regulate those systems. The converging evidence emphasizes the important role of these brain structures in understanding GAD.

The Fear Circuit



Note. This image depicts the location of important parts of the fear circuit including the amygdala, anterior cingulate cortex, prefrontal cortex, and locus coeruleus.

Researchers have also identified several other biological factors that can make people more likely to develop GAD. GAD has a moderate level of heritability. Heritability is a measure of how much genetic influences affect a characteristic, such as the development of GAD. If the heritability of a characteristic is 0, it means that genetics contributes nothing to the development of the trait. A heritability of 1 indicates that genetics completely explains the development of the trait. The heritability of GAD is .32 indicating the likelihood of developing GAD is explained in part by genetic influences. In addition, numerous studies have suggested that mutations in genes that impact the functioning of the brain's neurotransmitter can increase the likelihood of a person displaying the symptoms of GAD (Gottschalk & Domschke, 2017). Additionally, emerging research suggests that epigenetics—in which the environment exerts an influence on the way that genes are expressed—can provide a biological explanation for how environmental factors such as childhood trauma can lead to GAD in adulthood (Gottschalk & Domschke, 2017; Peedicayil, 2020).

Finally, recent research results provide evidence that the amounts and types of bacteria in the digestive tract of people with and without GAD differ (Jiang et al., 2018). Although focusing on gut bacteria may seem odd when discussing psychological disorders, there is ample evidence to suggest that the quality of these bacteria can impact mental health via gut-brain interactions (Chen, 2017). Giving further credence to this idea, Beibei Yang and colleagues (2019) reviewed a series of studies and concluded that treatments that improve the balance of digestive bacteria can significantly reduce anxiety levels in some participants (Yang et al., 2019).

Environmental Factors Influencing GAD

In addition to biological factors, sociocultural factors also can affect GAD development. For example, stress is a factor that increases the likelihood of many psychological disorders, GAD among them. Michael Zvolensky and his colleagues (2017) have suggested that because immigrants, people of color, LGBTQ individuals, and people with low socioeconomic status (SES) experience high degrees of stress (from, for example, discrimination and a lack of resources), they are at a greater risk for developing anxiety disorders.

Psychological factors also play a role in the development of GAD. Experiencing adversity in childhood and having overprotective parents have been associated with GAD. Individuals with GAD are also likely to pay more attention to worrisome information than individuals without GAD.

Cultural Issues

Researchers have found that people from numerous non-Western cultures experience GAD differently than do people from the West. Specifically, many non-Westerners experience the physiological symptoms of GAD more strongly than the psychological ones—the reverse of what is typically found in Americans and Western Europeans. For example, Americans typically describe their GAD using language describing psychological symptoms such as feeling scared or nervous. In contrast, Nepalis emphasize physiological symptoms such as dizziness and indigestion (Hoge et al., 2006). A similar focus on physical symptoms has been found in diverse cultures such as those of Hong Kong, Mexico, and the United Arab Emirates (Lewis-Fernández et al., 2010).

One symptom of GAD is worry about the occurrence of negative events in some aspect of daily life. Which aspects of daily life are the subject of excessive worry is likely to be influenced by cultural factors. Cultures that place a high degree of value on family relationships may lead to excessive worry about the death of a loved one. Cultures that place an emphasis on individual achievement may lead one to worry about their level of accomplishment or success. Clinicians' judgments of a person's symptoms must be informed by knowledge of cultural factors that provide the necessary context in which to understand a person's symptoms.

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